Which treatment this guide covers NCCP 00219a is first-line EGFR-mutated advanced NSCLC; 00219b is switch-maintenance after stable disease following first-line chemotherapy; 00219c is after failure of at least one previous chemotherapy regimen. Global NCCP eligibility requires a validated activating EGFR mutation in each setting. These are advanced-disease settings, not adjuvant treatment.

NCCP 00219 · VERSION 6

Erlotinib monotherapy

The usual dose is 150 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

Make treatment fit into daily life

Take at least 1 hour before or 2 hours after food. If a dose is missed, do not replace it; take the usual next dose. If you vomit within a few hours after taking it, do not repeat the dose. Ask pharmacy before changing smoking habits or adding medicines for stomach acid.

Before starting

Confirm the mutation, treatment setting, prescription, baseline tests, first review and emergency contact.

Taking the tablets

Take at least 1 hour before or 2 hours after food. If a dose is missed, do not replace it; take the usual next dose. If you vomit within a few hours after taking it, do not repeat the dose. Ask pharmacy before changing smoking habits or adding medicines for stomach acid.

Supplies and visits

Arrange the next prescription before tablets run out. Keep your blood-test and clinic appointments. The oral regimen itself does not require an infusion line or pump.

If treatment is held

Ask why, what must recover and when review is due. Only the team authorises restarting or changing dose.

At treatment change

Agree ongoing symptom care, scans, contraception and follow-up.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00219 v6 · official regimen
- EMA Tarceva product information
- NCCP SACT antiemetic guidance V6 (2025), oral tables
- NCCP NCIS antiemetic medicines V8 (2025); parenteral scope
- OPTIMAL primary PFS, Zhou et al. 2011
- OPTIMAL final OS, Zhou et al. 2015
- SATURN maintenance trial, Cappuzzo et al. 2010
- BR.21 previously treated NSCLC, Shepherd et al. 2005

Source reconciled; independent clinical, pharmacy and patient/PPI review pending